



Network Bulletin: March 2017

network bulletin

Important updates from UnitedHealthcare to health care professionals and facilities



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UnitedHealthcare respects the expertise of the physicians, health care professionals and their staff who participate in our network. Our goal is to support you and your patients in making the most informed decisions regarding the choice of quality and cost-effective care, and to support practice staff with a simple and predictable administrative experience. The Network Bulletin was developed to share important updates regarding UnitedHealthcare procedure and policy changes, as well as other useful administrative and clinical information.

Where information in this bulletin conflicts with applicable state and/or federal law, UnitedHealthcare follows such applicable federal and/or state law.

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Front & Center

Update: Demographic Data Attestation Requirement

To help provide UnitedHealthcare members with current information and meet Centers for Medicare & Medicaid Services (CMS) requirements, all care providers who are contracted with us must attest to the accuracy of their demographic information each quarter. This policy is outlined in the **2017 UnitedHealthcare Care Provider Administrative Guide (page 10)**.

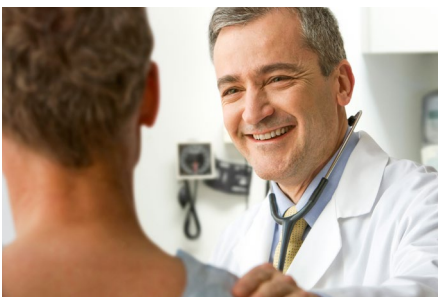
Up-to-date care provider directories can help connect you with members searching for a care provider, and accurate data also supports claims accuracy and timely reimbursement for you.

To make it easier for you to review and attest to the accuracy of your demographic data, we've made changes to the My Practice Profile app on Link — your gateway to UnitedHealthcare's online tools and resources. You must attest to the accuracy of your demographic data within 90 days of receiving access to the upgraded My Practice Profile app.

Link users will receive the upgraded app in phases through 2017. When it's available to your organization, authorized users can use it to review and update* the demographic information we have for you — including office addresses, hours and locations, phone and fax numbers, email and web addresses, languages spoken, ages and genders served, and much more.

We'll send you a notification email when you have access to the upgraded My Practice Profile app. To make sure you receive this email, please sign in to **UnitedHealthcareOnline.com** and go to My Profile to add or update your email address.

To learn more about the attestation requirement, please review the **Care Provider Demographic Data Attestation Frequently Asked Questions**. If you have questions about using My Practice Profile, call the UnitedHealthcare Connectivity Help Desk at 866-842-3278, option 3. If you have questions about the attestation requirement, please contact your Provider Advocate or call Provider Services at 877-842-3210.

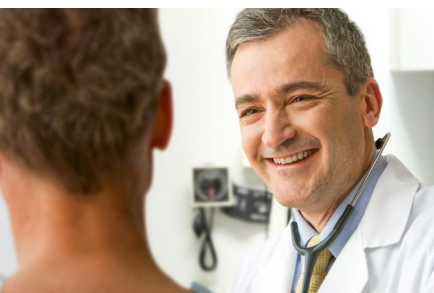


Front & Center

Update:
Demographic
Data Attestation
Requirement

*Care providers who participate with **UnitedHealthcare Community Plan of Hawaii** should not use My Practice Profile to update demographic information. Instead, please call 888-980-8728 to make demographic updates.

Delegated providers who submit **UnitedHealthcare Community Plan of Michigan** demographic updates through a separate process should not use My Practice Profile to update demographic information. Instead, please continue to submit those updates using your existing process.



Front & Center

Administrative Guide Update

In previous Network Bulletin articles, including an article in the **October 2016 issue**, we let you know the following National Drug Code (NDC) information must be included in your Commercial and Medicare Advantage claim and encounter data submissions:

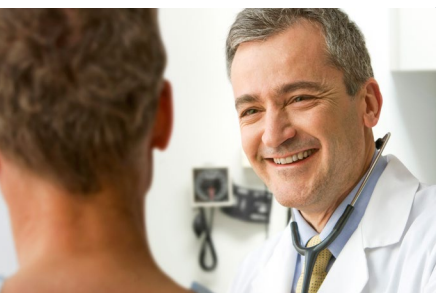
- Current NDC (National Drug Code) 11-digit number, NDC unit of measure (F2, GR, ML, UN) and NDC units dispensed (must be greater than 0) for all claims submitted with drug codes. The NDC information for the drug(s) administered must be entered in the 24D field of the CMS-1500 Form or the LIN03 and CTP04-05 segments of the HIPAA 837 Professional electronic form.

We want to let you know that Chapter 8: Claims Process on page 46-47 of the **2017 UnitedHealthcare Care Provider Administrative Guide** does not contain the complete NDC requirement, which states the unit of measure must be included and the units dispensed must be greater than 0.

The 2018 UnitedHealthcare Care Provider Administrative Guide will be updated to include the complete NDC requirement information. In the meantime, you need to follow the NDC requirement previously communicated for dates of service on or after Jan. 1, 2017. Information regarding the NDC requirement is available at UnitedHealthcareOnline.com > Claims & Payments > Claim Submission (on the left navigation menu) > **NDC Claim Submission**. A link to **frequently asked questions** is available there as well.

Pharmacy Update – Notice of Changes to Prior Authorization Requirements and Coverage Criteria for UnitedHealthcare Commercial and Oxford

A Pharmacy Bulletin outlining upcoming new or revised clinical programs and implementation dates is now available online for UnitedHealthcare Commercial. Go to **UnitedHealthcareOnline.com** > Tools & Resources > Pharmacy Resources. Then select Clinical Programs and scroll down to the Resources box to access the updates.



Front & Center

2017 Updates to Mammography Billing Codes

We know that you might have some questions about this year's changes to mammography CPT® and Healthcare Common Procedure Coding System (HCPCS) codes. For dates of service Jan. 1, 2017 and later, here's a list of the codes that were added, the codes with updated definitions and the codes that have been removed. The updated mammography codes include computer-aided detection (CAD) services starting Jan. 1, 2017:

New CPT Codes:

- These code definitions include CAD services
 - 77065, 77066 and 77067

Updated CPT codes:

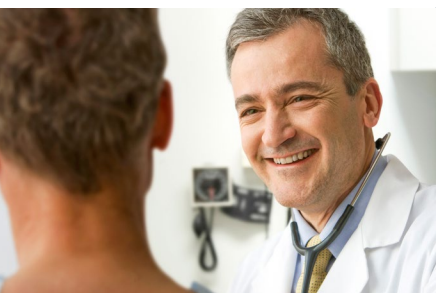
- These code definitions include CAD services
 - G0202, G0204 and G0206

Removed CPT codes:

- These codes aren't valid for dates of service after Dec. 31, 2016
 - 77055, 77056 and 77057 (mammography)
 - 77051 and 77052 (CAD)

Although the code numbers and descriptions changed, we're working to keep the reimbursement you get as close as possible to your previous reimbursement levels for these services. Your participation agreement payment appendix will have your specific fee schedule information and reimbursement depends on your fee schedule.

We're updating our billing procedures and reimbursements to include these code changes. If you have questions related to these changes, please contact your local Network Management representative.



Front & Center

Referral Required Message to Display on Eligibility and Benefit (271) EDI Response Transactions

UnitedHealthcare will add an Electronic Data Interchange (EDI) referral indicator to eligibility response (271) transactions beginning March 1, 2017 for all lines of business.* Eligibility responses will then indicate if a member is enrolled in a plan requiring a referral from their Primary Care Physician (PCP).

If a member is enrolled in a plan requiring a referral from their PCP before seeing a network specialist, the following message will appear notifying the care provider: *PCP to submit a specialist referral*. **Referrals** should be requested electronically as a 278 EDI transaction.

If you don't see a referral requirement message on your eligibility responses for applicable plan members on or after the effective date, contact your software vendor or clearinghouse. **We have informed our trading partners that action will be needed to display the required referral message in systems used by their customers.**

The following steps outline the changes UnitedHealthcare is making to the 271 transaction (005010X279A1):

1. EB11 Authorization or Certification Indicator in the 2110c and 2110d loop include the Y value to indicate preauthorization/ notification/referral is required at the plan level.
2. For the 2110c and 2110d, MSG segment, include the referral reminder message: "PCP to submit a specialist referral."

Following is a sample 271 response displaying the referral required value and message segment:

EB*130*C1*UNITEDHEALTHCARE NAVIGATE*****Y~**
MSG*PCP TO SUBMIT A SPECIALIST REFERRAL



If you have questions, contact UnitedHealthcare EDI Support at 800-842-1109 or go online to our **EDI Transaction Support Form**.

*Includes UnitedHealthcare Commercial, UnitedHealthcare Community Plan and UnitedHealthcare Medicare & Retirement; excludes UnitedHealthcare Military & Veterans and UnitedHealthcare Oxford.



UnitedHealthcare Commercial

UnitedHealth Premium® Designation Program Reminders and Updates

Effective Date for Public Designation Display

The physician notification letters for the UnitedHealth Premium designation program were mailed in December 2016 to eligible physicians. As noted in these letters, the effective date for the new designations is March 30, 2017. Also beginning March 30, 2017, ophthalmology will no longer be included in the Premium program.

Reconsideration Requests for 2017

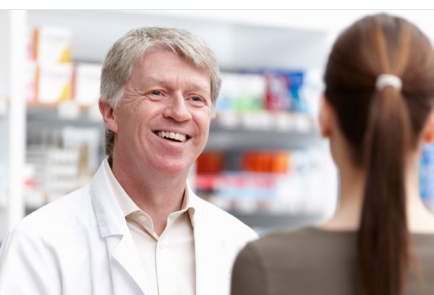
We'll continue to accept physician reconsideration requests as long as they are submitted on or before March 31, 2017. Please make your request by signing in to [UnitedHealthcareOnline.com](#) > Quick Links > UnitedHealth Premium > Premium Reconsideration > Request Reconsideration.

Directories—Designation Display Changes

The rollout of the new UnitedHealth Premium designations will occur in two phases beginning on March 30, 2017.

1. March 30, 2017

- The new text for the updated physician designations will display without the heart symbols.
- Physicians receiving a designation of Premium Care Physician (physicians who meet quality and cost efficiency criteria) will be identified in directories with the following Tier 1 icon and text:  Tier 1 Provider.



UnitedHealthcare Commercial

UnitedHealth Premium® Designation Program Reminders and Updates

2. Second Quarter 2017

- The new Premium designation heart symbols will display to indicate the designation of Premium-eligible physicians.

♥♥ Premium Care Physician

♥♡ Quality Care Physician

♡♡ Quality Not Evaluated

♡♡ Does Not Meet Quality

-  Tier 1 Provider will display for members with tiered benefit plans.



For more information about the UnitedHealth Premium program, please go to UnitedHealthcareOnline.com > Quick Links > **UnitedHealth Premium** or call 866-270-5588.



UnitedHealthcare Commercial

Harken Health Offers Free ERA/EFT through InstaMed

Harken Health is working with a new vendor, InstaMed, to deliver claim payments as electronic remittance advice (ERA) and electronic funds transfer (EFT).

To receive Harken Health payments as a free, secure ERA/EFT transaction, register at instamed.com/eraeft. Funds will be deposited directly into your designated bank account and will include the TRN Reassociation Trace Number in accordance with Council for Affordable Healthcare Quality (CAQH) CORE Phase III Operating Rules for HIPAA standard transactions.

Benefits of ERA/EFT include:

- Accelerated access to funds
- Convenient and secure payments
- Reduced administrative costs by eliminating paper checks and remittances
- No disruption to your current workflow — there is an option to have ERAs routed to your existing clearinghouse

Register today by:

- Visiting instamed.com/eraeft
- Calling 866-945-7990
- Emailing connect@instamed.com



UnitedHealthcare Commercial

Notification/Prior Authorization Requirement for Certain Office-Based Procedures to be Covered in Other Sites of Service — Effective April 1, 2017

As a reminder, beginning with April 1, 2017 dates of service, we'll require notification/prior authorization for certain procedures to be covered in settings other than a physician's office. This requirement, which was first announced in the **January 2017 Network Bulletin**, was developed with care experiences and out-of-pocket costs for our members in mind. It will apply to the following UnitedHealthcare Commercial plans, including Exchange plans, in states where we already conduct site of service medical necessity reviews.

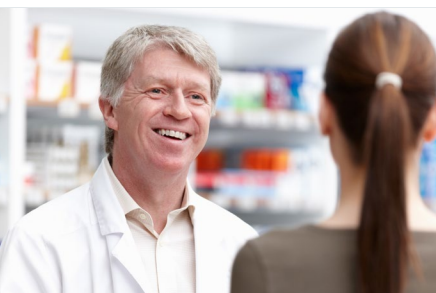
- Golden Rule Insurance Company (group 902667)
- Mid-AtlanticMD Healthplan Individual Practice Association, Inc. (M.D. IPA) or Optimum Choice, Inc. plans
- Neighborhood Health Partnership*
- UnitedHealthcare of the River Valley*
- UnitedHealthcare Oxford*
- UnitedHealthcare
- UnitedHealthcare Life Insurance Company (group 755870)

**Neighborhood Health Partnership, UnitedHealthcare Oxford and UnitedHealthcare of the River Valley plans already require prior authorization requests to evaluate whether it's medically necessary for these procedures to be provided in any setting other than a physician's office. Site of service will now be reviewed as part the existing prior authorization review process for these procedures.*

For dates of service on or after April 1, 2017, care providers must provide notification for the following procedures to be covered in a site of service other than a physician's office. *Please note that notification must be provided when these procedures are performed in a physician's office (Place of Service code 11) with an accompanying facility charge:*

Dermatologic	11402	11422	11403	11406	
Neurologic	62270	62320	62321	62322	62323
	64633	64635			
Muscular/Skeletal	27096	64479	64483	64490	64493
Respiratory	31579				

Important clarification: In previous communications about this requirement, CPT code 11100 was included. Notification/prior authorization will not be required for CPT code 11100.



UnitedHealthcare Commercial

Notification/Prior Authorization Requirement for Certain Office- Based Procedures to be Covered in Other Sites of Service – Effective April 1, 2017

When we receive notification, we'll determine if the member's benefit plan requires covered services to be medically necessary. If so, we'll evaluate the medical necessity of the site of service as part of our prior authorization process. Coverage determinations will take into account whether the patient has a need for more intensive services and whether the office has the equipment necessary to deliver the service.

In many cases, a physician's office is medically appropriate for patients and may offer them more convenient care experiences, as well as meaningful savings on out-of-pocket costs compared with other sites of service.

While notification/prior authorization is not required for these procedures to be covered in a physician's office, if you bill for a site of service other than a physician's office without following the notification/prior authorization process, claims will be denied. Members cannot be billed for services that are denied due to lack of notification.

UnitedHealthcare continues to focus on site of service as part of our clinical review process, as we work toward the Triple Aim of better care, better health and better costs for our members. Site of service medical necessity reviews align with our member benefit plan requirements that services, including sites of service, be medically necessary, including cost-effective.

We encourage you to take advantage of our online process for providing notification to save you time. Please go to UnitedHealthcareOnline.com and sign into Link to get started. Or call the number on the back of your patient's health plan ID card.



For more information, please go to **UnitedHealthcareOnline.com** > Tools & Resources > Policies, Protocols and Guides > Protocols > Notification/Prior Authorization Requirement for Certain Office-Based Procedures to be Covered in Other Sites of Service.

If you have questions, please contact your local Network Management representative or call the Provider Services number on the back of the member's ID card. Thank you.



UnitedHealthcare Commercial

UnitedHealthcare Medical Policy, Drug Policy and Coverage Determination Guideline Updates



For complete details on the policy updates listed in the following table, please refer to the **February 2017 Medical Policy Update Bulletin** at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Medical & Drug Policies and Coverage Determination Guidelines > Medical Policy Update Bulletin](#).

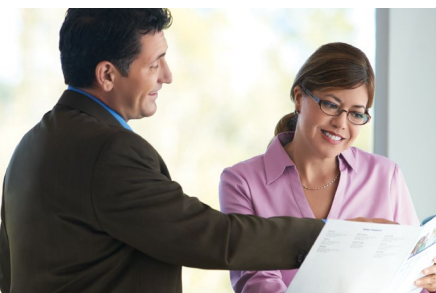
Policy Title	Policy Type	Effective Date
NEW		
Office Based Program	URG	April 1, 2017
Spinraza™ (Nusinersen)	Drug	April 1, 2017
UPDATED/REVISED		
Bariatric Surgery	Medical	March 1, 2017
Bone or Soft Tissue Healing and Fusion Enhancement Products	Medical	March 1, 2017
Deep Brain and Cortical Stimulation	Medical	March 1, 2017
Durable Medical Equipment, Orthotics, Ostomy Supplies, Medical Supplies and Repairs/Replacements	CDG	March 1, 2017
Epidural Steroid and Facet Injections for Spinal Pain	Medical	March 1, 2017
Fecal Calprotectin Testing	Medical	Feb. 1, 2017
Home Health Care	CDG	March 1, 2017
Home Hemodialysis	Medical	March 1, 2017
Infliximab (Remicade® and Inflectra™)	Drug	April 1, 2017
Omnibus Codes	Medical	March 1, 2017
Outpatient Cardiovascular Telemetry	Medical	Feb. 1, 2017
Surgical and Ablative Procedures for Venous Insufficiency and Varicose Veins	Medical	Feb. 1, 2017
Transcranial Magnetic Stimulation	Medical	Feb. 1, 2017
RETIRED		
Intermittent Intravenous Insulin Therapy	Medical	Feb. 1, 2017

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.



UnitedHealthcare Commercial Reimbursement Policies

Unless otherwise noted, the following reimbursement policies apply to services reported using the 1500 Health Insurance Claim Form (CMS-1500) or its electronic equivalent or its successor form. UnitedHealthcare reimbursement policies do not address all factors that affect reimbursement for services rendered to UnitedHealthcare members, including legislative mandates, member benefit coverage documents, UnitedHealthcare medical or drug policies, and the UnitedHealthcare Care Provider Administrative Guide. Meeting the terms of a particular reimbursement policy is not a guarantee of payment. Once implemented, the policies may be viewed in their entirety at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Reimbursement Policies-Commercial](#). In the event of an inconsistency between the information provided in the Network Bulletin and the posted policy, the posted policy prevails.



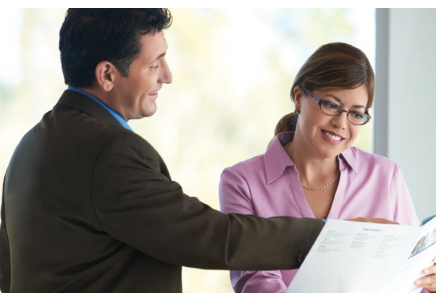
UnitedHealthcare Commercial Reimbursement Policies

Retirement of UnitedHealthcare Physical Medicine & Rehabilitation: Supervised Modalities Policy

The current UnitedHealthcare Physical Medicine & Rehabilitation: Supervised Modalities Policy reimbursement policy for Commercial plans will be retired in the second quarter of 2017. The two CPT codes listed in the retired policy (97010, 97014) will be supported by other reimbursement policies based on their assigned status code indicator on the Centers for Medicare & Medicaid Services (CMS) National Physician Fee Schedule (NPFS) Relative Value File.

CPT code 97010 will be supported by the UnitedHealthcare B Bundle reimbursement policy. CPT code 97010 is assigned a status indicator of "B". Per the public use file that accompanies the NPFS Relative Value File, the following is stated for status code "B": "Payment for covered services are always bundled into payment for other services not specified. If RVUs are shown, they are not used for Medicare payment. If these services are covered, payment for them is subsumed by the payment for the services to which they are incident. (An example is a telephone call from a hospital nurse regarding care of a patient)." Consistent with CMS, UnitedHealthcare does not separately reimburse for specific codes assigned a status code "B" on the NPFS Relative Value File indicating a bundled procedure.

CPT code 97014 will be supported by the UnitedHealthcare Replacement Codes reimbursement policy. CPT code 97014 is assigned a status indicator of "I". Per the public use file that accompanies the NPFS Relative Value File, the following is stated for status code "I": "Not valid for Medicare purposes. Medicare uses another code for reporting of, and payment for, these services." Consistent with CMS, UnitedHealthcare does not separately reimburse for codes assigned a status code "I" on the NPFS Relative Value File, indicating another code (replacement code) is used to report the procedure or service and that replacement code has an assigned RVU.



UnitedHealthcare Commercial Reimbursement Policies

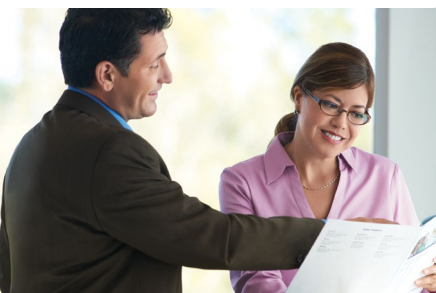
Revised Intensity-Modulated Radiation Therapy (IMRT) Policy

Effective for claims with dates of service on or after June 1, 2017, UnitedHealthcare will enhance the IMRT Policy to further align with the American Society of Therapeutic Radiation Oncology (ASTRO) coding guidelines.

The ASTRO 2016 Radiation Oncology Coding Resource states, "Certain ancillary services performed as part of the development of an IMRT treatment plan are included in the payment for CPT code 77301 and may not be billed separately, regardless if billed on the same or a different date of service. These services include CT imaging for treatment planning (77014), treatment simulations (77280-77290), external beam isodose planning (77295, 77306, and 77307), special teletherapy port plan (77321), special dosimetry (77331) and medical physics consultation (77370)."

The current IMRT policy does not allow reimbursement for an IMRT simulation (CPT codes 77280, 77285, 77290) when performed on the same tumor, regardless if billed on the same or a different date of service as IMRT plan code 77301. The enhanced IMRT policy will not allow reimbursement for radiation therapy services (CPT codes 77014, 77295, 77306, 77307, 77321, 77331, 77370) when performed on the same tumor, regardless if billed on the same or a different date of service as IMRT plan code 77301.

This new policy will apply to services reported using the 1500 Health Insurance Claim Form (CMS-1500) or its electronic equivalent or its successor form.



UnitedHealthcare Commercial Reimbursement Policies Required Modifiers for Biosimilar Drugs

UnitedHealthcare will begin requiring biosimilar biological products to include a modifier that identifies the manufacturer of the specific product for dates of service on or after June 1, 2017. This coding edit is consistent with the Centers for Medicare & Medicaid Services (CMS) and will be addressed in UnitedHealthcare's Procedure to Modifier Policy. The corresponding modifier requirement will be applicable as additional biosimilar procedure codes and/or modifiers are created. Biosimilar drug codes reported without the modifier will be denied. Claims that are denied can be resubmitted with the appropriate modifier.



Additional details and the procedure codes this requirement applies to are available on the CMS website at cms.gov/Medicare/Medicare-Fee-for-Service-Part-B-Drugs/McrPartBDrugAvgSalesPrice/Part-B-Biosimilar-Biological-Product-Payment.html.

Biosimilar HCPCS Code	Product Brand Names	Corresponding Required Modifier
Q5101 – Injection, Filgrastim (G-CSF), Biosimilar, 1 microgram	Zarxio	ZA – Novartis/Sandoz
Q5102 – Injection, infliximab, biosimilar 10 mg	Inflectra	ZB – Pfizer/Hospira



UnitedHealthcare Community Plan

New Policy for Exondys 51

Effective April 1, 2017, UnitedHealthcare Community Plan will implement a new drug policy for pre-service and post-service reviews of Exondys 51™. Any members new to the therapy or who are continuing in therapy will need to meet the guidelines outlined in the policy.

Exondys 51 (eteplirsen) is indicated for treatment of patients with Duchenne muscular dystrophy (DMD) in patients with a confirmed mutation of the DMD gene that is amenable to exon 51 skipping.

This new policy applies to UnitedHealthcare Community Plan members in the following states:

Arizona, Delaware, Florida, Kansas,* Louisiana, Mississippi, Nebraska, New Jersey, New Mexico, New York, Ohio, Pennsylvania, Rhode Island, Tennessee, Texas and Washington.

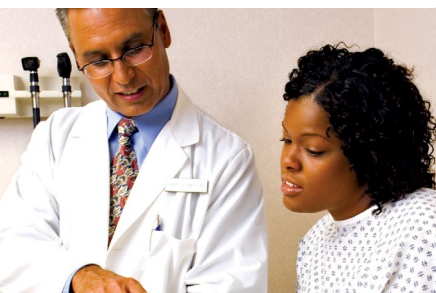
*Jan. 11, 2017 effective date for Kansas.

The policy does not apply to the following UnitedHealthcare Community Plan plans:

- UnitedHealthcare Dual Complete® plans
- Medicare-Medicaid Enrollees (MME)
- UnitedHealthcare Connected® for MyCare Ohio
- UnitedHealthcare Connected® (in Texas)
- UnitedHealthcare Senior Care Options (in Massachusetts)



If you have any questions, please call the Provider Service number on the back of the member's ID card. For more information, go to **UnitedHealthcareOnline.com**.



UnitedHealthcare Community Plan

Changes in Advance Notification and Prior Authorization Requirements

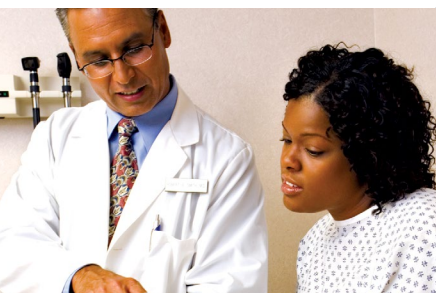
Code Replacements to Prior Authorization

Effective for dates of service on or after Jan. 1, 2017 for UnitedHealthcare Community Plans in Arizona, Florida and Wisconsin, new codes will be added to prior authorization as a result of the American Medical Association (AMA) 2017 annual update to the Current Procedure Terminology.

Category	Deleted Codes	Replacement Codes Added to Prior Authorization
Outpatient Therapy	97001	97161 97162 97163
	97002	97164
	97003	97165 97166 97167
	97004	97168

The most up-to-date Advance Notification lists are available online:

- UnitedHealthcare Community Plan – UHCommunityPlan.com > For Health Care Professionals > Select your state.



UnitedHealthcare Community Plan

Preferred Drug List Updates for Second Quarter 2017

UnitedHealthcare Community Plan's Preferred Drug List (PDL) is updated quarterly by our Pharmacy and Therapeutics Committee. Please review the changes and update your references as necessary.



You may also view the changes at UHCommunityPlan.com > For Health Care Professionals > Select your state > Pharmacy Program.

We provided a list of available alternatives to members whose current treatment includes a medication removed from the PDL. Please provide affected members with a prescription for a preferred alternative in one of these ways:

- Call or fax the pharmacy
- Use e-Script
- Write a new prescription and give it to the member



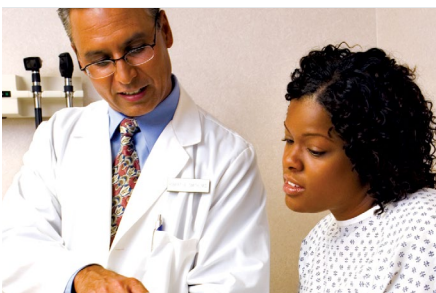
If a preferred alternative is not appropriate, call 800-310-6826 for prior authorization for the member to remain on their current medication.

Changes will be effective April 1, 2017

PDL Additions

Brand Name	Generic Name	Comments
Hypercare 15%	Aluminum chloride topical solution	Indicated for treating and preventing hyperhidrosis.
Lovaza* capsule	Omega 3 acid ethyl esters	Indicated for treating hypertriglyceridemia. Prior authorization required.
Orfadin capsule	Nitisinone	Indicated for treating hereditary tyrosinemia type I. Prior authorization required. Available through specialty pharmacy.
Pin-X Chewable tablet	Pyrantel pamoate	Indicated for treating enterobiasis (pinworm infection).
Pin-X Suspension	Pyrantel pamoate	Indicated for treating enterobiasis (pinworm infection).
Reese's Pinworm Medicine Tablet	Pyrantel pamoate	Indicated for treating enterobiasis (pinworm infection).
Reese's Pinworm Medicine Suspension	Pyrantel pamoate	Indicated for treating enterobiasis (pinworm infection).
Invokamet XR tablet	Canagliflozin/metformin extended release	Indicated for treating type 2 diabetes mellitus. Step therapy applies.
Synjardy XR tablet	Empagliflozin/metformin extended release	Indicated for treating type 2 diabetes mellitus. Step therapy applies.
Xiidra ophthalmic solution	Lifitegrast	Indicated for treating dry eye disease. Prior authorization required.

*Only generics are covered.



UnitedHealthcare Community Plan

Preferred Drug List Updates for Second Quarter 2017

PDL Modifications

Brand Name	Generic Name	Comments
Albenza tablet	Albendazole	Prior authorization required. Indicated for treating Cysticercosis, Hydatid cyst and Neurocysticercosis.
Copegus* 200 mg tablet	Ribavirin	Prior authorization removed. Specialty pharmacy required. Indicated for treating chronic hepatitis C infection.
Rebetol* 200 mg capsule	Ribavirin	Prior authorization removed. Specialty pharmacy required. Indicated for treating chronic hepatitis C infection.
ProAmantine* tablet	Midodrine	Step therapy removed. Indicated for treating orthostatic hypotension.

*Only generics are covered.

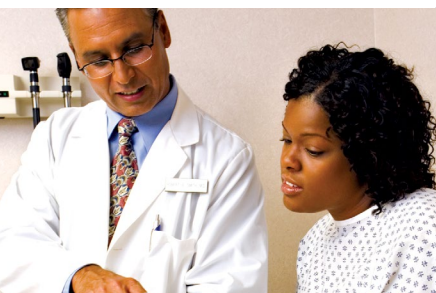
PDL Deletions

Brand Name	Generic Name	Comments
Atralin* Gel 0.05%	Tretinoin 0.05% gel	Alternative formulations are available including tretinoin gel 0.025%, tretinoin gel 0.01%, tretinoin cream 0.025%, tretinoin cream 0.05% and tretinoin cream 0.1%. Current users will not be grandfathered.
Dolophine* 5 mg tab 10 mg tab 5 mg/5 ml 10 mg/5 ml	Methadone	Alternative agents are available on the preferred drug list including morphine sulfate ER, fentanyl patch, oxycodone ER and Zohydro ER. Prior authorization required. Current users will be grandfathered.
Drysol 20%	Aluminum chloride topical solution	An alternative formulation is available including Hypercare 15%.
Mydracyl	Tropicamide ophthalmic solution	Tropicamide ophthalmic solution should only be used for diagnostic purposes and eye examinations. Various alternative ophthalmic preparations are available on the preferred drug list.
Restasis ophthalmic emulsion	Cyclosporine ophthalmic emulsion	An alternative agent is available including Xiidra ophthalmic solution. Prior authorization is required. Current users will not be grandfathered.

*Only generics are covered.



If you have questions, call UnitedHealthcare Community Plan's Pharmacy Department at 800-310-6826.



UnitedHealthcare Community Plan

UnitedHealthcare Community Plan Medical Policy & Coverage Determination Guideline Updates

For complete details on the policy updates listed in the following table, please refer to the **February 2017 Medical Policy Update Bulletin** at [UHCCCommunityPlan.com > For Health Care Professionals > Select Your State > Provider Information > UnitedHealthcare Community Plan Medical Policies and Coverage Determination Guidelines](#).

Policy Title	Policy Type	Effective Date
NEW		
Exondys 51™ (Eteplirsen)	Drug	April 1, 2017
Ocrevus® (Ocrelizumab)	Drug	April 1, 2017
Probuphine® (Buprenorphine)	Drug	April 1, 2017
Respiratory Interleukins (IL) Policy	Drug	April 1, 2017
Spinraza™ (Nusinersen)	Drug	May 1, 2017
UPDATED/REVISED		
Bariatric Surgery	Medical	March 1, 2017
Bone or Soft Tissue Healing and Fusion Enhancement Products	Medical	April 1, 2017
Deep Brain and Cortical Stimulation	Medical	March 1, 2017
Durable Medical Equipment, Orthotics, Ostomy Supplies, Medical Supplies and Repairs/Replacements	CDG	March 1, 2017
Epidural Steroid and Facet Injections for Spinal Pain	Medical	April 1, 2017
Fecal Calprotectin Testing	Medical	Feb. 1, 2017
Home Health Care	CDG	April 1, 2017
Home Hemodialysis	Medical	April 1, 2017
Hospice (Applies to the State of Louisiana Only)	CDG	March 1, 2017
Mechanical Stretching Devices	Medical	March 1, 2017
Omnibus Codes	Medical	April 1, 2017
Outpatient Cardiovascular Telemetry	Medical	Feb. 1, 2017
Surgical and Ablative Procedures for Venous Insufficiency and Varicose Veins	Medical	Feb. 1, 2017
Transcranial Magnetic Stimulation	Medical	Feb. 1, 2017
RETIRED		
Intermittent Intravenous Insulin Therapy	Medical	Feb. 1, 2017

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.



UnitedHealthcare Medicare Solutions

New Medicare Advantage PCP Change Request Form

At UnitedHealthcare, we continuously look for ways to help make the access to health care services simpler for our members. That's why we make it easier for our members to change primary care providers (PCPs). As a PCP, you can help your members with this process.



PCPs in New York, New Jersey, Massachusetts, Connecticut and Rhode Island can now download our new Medicare Advantage Primary Care Provider Change Request Form. You can find the new request form at **UnitedHealthcareOnline.com** > Tools and Resources > Forms > Administrative > PCP Change Request Form. Please help members by filling out the form with them and faxing it to UnitedHealthcare Member Services at 844-881-4857. Please include a fax cover letter with your form in case we need to contact your provider's office with questions.

Members also can call the Member Services telephone number on the back of their member ID card to ask for a PCP change. Changes submitted to UnitedHealthcare will generally be processed within one to two business days after the form is faxed to member services and will be effective on the first day of the following month. The member will be notified of the change when they receive a new member ID card in the mail.



UnitedHealthcare Medicare Solutions

UnitedHealthcare Medicare Advantage Coverage Summary Updates



For complete details on the policy updates listed in the following table, please refer to the **February 2017 Medicare Advantage Coverage Summary Update Bulletin** at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > UnitedHealthcare Medicare Advantage Coverage Summaries > Update Bulletin](#).

Policy Title
UPDATED/REVISED (Approved on Jan. 17, 2017)
Allergy Testing and Allergy Immunotherapy
Ambulance Services
Blepharoplasty and Related Procedures
Brachytherapy Procedures
Cardiovascular Diagnostic Procedures
Chelation Therapy
Complementary and Alternative Medicine
Cosmetic and Reconstructive Procedures
Dialysis Services
Evaluation and Management Services
Gastroesophageal and Gastrointestinal (GI) Services and Procedures
Glaucoma Surgical Treatments
Impotence Treatment
Infusion Pump Therapy
Laser Procedures
Neurologic Services and Procedures
Pain Management and Pain Rehabilitation
Spine Procedures
Transcatheter Heart Valve Procedures
Varicose Veins Treatment and Other Vein Embolization Procedures

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.



UnitedHealthcare Medicare Solutions

UnitedHealthcare Medicare Advantage Policy Guideline Updates



The following UnitedHealthcare Medicare Advantage Policy Guidelines have been updated to reflect the most current clinical coverage rules and guidelines developed by the Centers for Medicare & Medicaid Services (CMS). The updated policies are available for your reference at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > UnitedHealthcare Medicare Advantage Policy Guidelines](#).

Policy Title
UPDATED/REVISED (Approved on Jan. 11, 2017)
Bone (Mineral) Density Studies (NCD 150.3)
Breast Reconstruction Following Mastectomy (NCD 140.2)
Collagen Meniscus Implant (NCD 150.12)
Continuous Positive Airway Pressure (CPAP) Therapy for Obstructive Sleep Apnea (OSA) (NCD 240.4) and Other Respiratory Assist Devices (RAD)
Corus [®] CAD (Coronary Artery Disease)
Electrical Stimulation (ES) and Electromagnetic Therapy for the Treatment of Wounds (NCD 270.1)
Heartsbreath Test for Heart Transplant Rejection (NCD 260.10)
Heat Treatment, Including the Use of Diathermy and Ultrasound for Pulmonary Conditions (NCD 240.3)
Home Health Visits to a Blind Diabetic (NCD 290.1)
Human Tumor Stem Cell Drug Sensitivity Assays (NCD 190.7)
Incontinence Control Devices (NCD 230.10)
Insertion of Posterior Spinous Process Device
Knee Orthoses
Low Frequency, Non-Contact, Non-Thermal Ultrasound
Lumbar Artificial Disc Replacement (LADR) (NCD 150.10)
Lung Cancer Screening with Low Dose Computed Tomography (LDCT) (NCD 210.14)
Medical Nutrition Therapy (NCD 180.1)
Mobility Devices (Non-Ambulatory) and Accessories
Obsolete or Unreliable Diagnostic Tests (NCD 300.1)



UnitedHealthcare Medicare Solutions

UnitedHealthcare Medicare Advantage Policy Guideline Updates

Policy Title
Outpatient Intravenous Insulin Treatment (OIVIT) (NCD 40.7)
Pharmacogenomic Testing for Warfarin Response (NCD 90.1)
Postural Drainage Procedures and Pulmonary Exercises (NCD 240.7)
Sacral Nerve Stimulation for Urinary Incontinence (NCD 230.18)
Sweat Test (NCD 190.5)
Thermal Intradiscal Procedures (TIPs) (NCD 150.11)
Treatment of Actinic Keratosis (NCD 250.4)
Uroflowmetric Evaluations (NCD 230.2)

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.



UnitedHealthcare Military & Veterans

TRICARE Expands Preventive Care Coverage

We wanted to let you know that TRICARE added preventive services and screenings to their coverage. This helps you deliver more preventive care to TRICARE beneficiaries at no out-of-pocket cost to them. Starting Jan. 1, 2017, TRICARE covers:

- One yearly preventive office visit for TRICARE Prime beneficiaries ages 6 and older; beneficiaries ages 5 and younger will still use TRICARE's **well-child coverage**
- Physicals required for school enrollment with no age limit
- Annual **well-woman exams** at no cost for beneficiaries younger than 65
- BRCA1 or BRCA2 genetic counseling at no cost for women at a high risk for breast cancer; BRCA1 or BRCA2 genetic testing is covered for women who meet the **laboratory developed test** coverage guidelines
- Pap tests beginning at age 21
- Stool DNA testing (such as Cologuard®) once every three years starting at age 50 for beneficiaries with an average risk for colon cancer
- Computed tomographic colonography (virtual colonoscopy) once every five years for beneficiaries ages 50 and older with an average risk for colon cancer
- Cancer screenings (testicular, skin, thyroid, mouth and pharyngeal) during a covered office visit



UnitedHealthcare Military & Veterans

TRICARE Expands Preventive Care Coverage

TRICARE also covers the following screenings and services for TRICARE Standard and Extra beneficiaries with no out-of-pocket costs when delivered during a covered health promotion and disease prevention exam (an exam that also includes a vaccination or cancer screening):

- Prenatal screenings
- Tuberculosis (TB) screening
- Rubella antibodies
- Hepatitis B and C virus screening
- Diabetes mellitus (type 2) screening
- HIV infection screening
- Syphilis, gonorrhea and chlamydia screenings
- Cholesterol tests and blood pressure screenings
- Osteoporosis screening
- Intensive behavioral counseling for sexually transmitted infections (STI)

TRICARE made these changes to help promote preventive care and to better align with the recommendations of the United States Preventive Services Task Force (USPSTF) and the Health Resources and Services Administration's (HRSA) guidelines.

Beneficiary Costs

TRICARE Prime beneficiaries don't need referrals or prior authorizations to get preventive care from their primary care manager or any TRICARE-network provider.* TRICARE Prime beneficiaries won't have out of pocket costs.

TRICARE Standard beneficiaries have no cost share or copay when their preventive services are part of a health promotion and disease prevention exam.

If you have any questions about delivering preventive care to TRICARE beneficiaries, please contact your Provider Advocate or call us at **877-988-9378**. Thank you.

*Active duty service members always need prior authorization when getting care from a network provider.



UnitedHealthcare Military & Veterans

New Navigation and Improved Functionality Highlight UHCMilitaryWest.com Updates

We're starting 2017 with a fresh look and enhanced navigation for **UHCMilitaryWest.com**. Our site now:

- Works better with your mobile devices
- Has a new provider search tool called "Find Care" with new, easier navigation

UHCMilitaryWest.com will continue to bring you the most current and relevant information to help you serve TRICARE beneficiaries. Be sure to add UHCMilitaryWest.com to your favorites and check back often.

If you don't already have an account, register for your secure UHCMilitaryWest.com account today. Once registered, you can manage your claims and prior authorization requests, and sign up to receive email alerts when important TRICARE documents are available to view online.



If you have questions about using UHCMilitaryWest.com or registering for an account, please visit our online user guides at **UHCMilitaryWest.com > Reference Guides**.



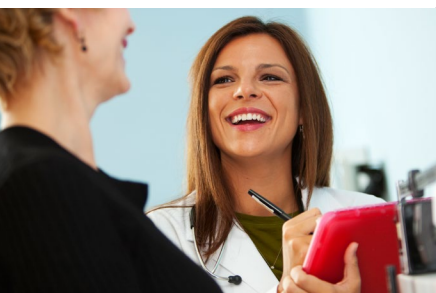
Doing Business Better

UnitedHealthcare Credentialing Plan 2017-2018 Summary of Changes

UnitedHealthcare reviews and updates our Credentialing Plan at least every two years to help ensure compliance with all state and regulatory requirements and accreditation requirements of the National Committee for Quality Assurance (NCQA).

The 2017 Credentialing Plan and State and Federal Regulatory Addendum will take effect on June 1, 2017. Here are some of the key changes:

- Expanded definition of Material Restriction to include same or similar specialty reference.
- Revised language to include discretionary ability of the National Credentialing Center staff to recommend contract termination of a LIP whose recredentialing application remains incomplete.
- Revised application completion timeframe to read 180 days or in accordance with credentialing authorities in a shorter time frame.
- Expanded facilities section to include evidence from an accrediting body or a CMS quality state review not older than three years or CMS letter of passed inspection.
- Expanded practitioner rights section to include the format for submitting practitioner corrections and where to submit those corrections.
- Revised the On-going Monitoring section to include a description of the appeal process for adverse events based on credentialing or sanctions monitoring decisions.



Doing Business Better

Federal Requirements for Advance Directives

The federal Patient Self-Determination Act (PSDA) gives individuals the legal right to make choices about their medical care in advance of incapacitating illness or injury through an advance directive.

Under this act, physicians and other health care professionals, including hospitals, skilled nursing facilities, hospices, home health agencies and others must provide written information to patients about state law, advance treatment directives, the patients' right to accept or refuse treatment and your own policies regarding advance directives. UnitedHealthcare also informs our members about advance directives through our member handbooks and other communications.

Medicare Members' Rights and Responsibilities

The Centers for Medicare & Medicaid Services offers the booklet, *Your Medicare Rights and Protections*. Care providers can direct members to the booklet at [medicare.gov/Pubs/pdf/11534.pdf](https://www.medicare.gov/Pubs/pdf/11534.pdf). Members also can order a free copy by calling 800-MEDICARE (800-633-4227) or TTY (877-486-2048), 24 hours a day, 7 days a week. CMS also provides information on Medicare rights and protections for members at [medicare.gov/claims-and-appeals/medicare-rights/medicare-rights-overview.html](https://www.medicare.gov/claims-and-appeals/medicare-rights/medicare-rights-overview.html). For more information on Medicare Members' Rights and Responsibilities, go online to aarpmedicareplans.com/health-plans/medicare-advantage-plans/resources-plan-material/ma-medicare-forms/medicare-rights-center.html.



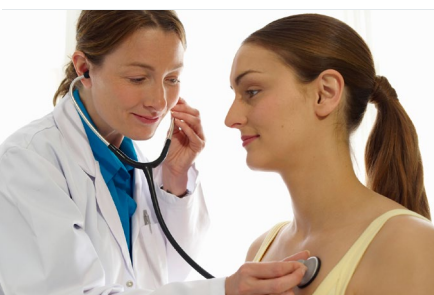
UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates



For complete details on the policy updates listed in the following table, please refer to the **February 2017 Policy Update Bulletin** at **OxfordHealth.com > Providers > Tools & Resources > Medical Information > Medical and Administrative Policies > Policy Update Bulletin**.

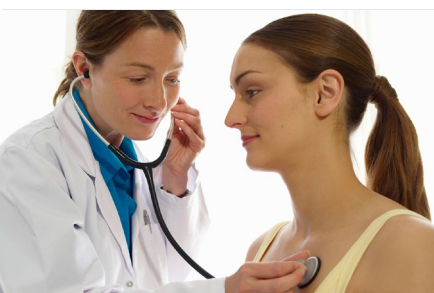
Policy Title	Policy Type	Effective Date
NEW		
Office Based Program	Clinical	April 1, 2017
Spinraza™ (Nusinersen)	Clinical	April 1, 2017
UPDATED/REVISED		
Actemra® (Tocilizumab) Injection for Intravenous Infusion	Clinical	Feb. 1, 2017
Assistant Surgeon	Reimbursement	Feb. 1, 2017
Assisted Administration of Clotting Factors and Coagulant Blood Products	Clinical	Feb. 1, 2017
Autism	Administrative	Feb. 1, 2017
Bariatric Surgery	Clinical	March 1, 2017
Behavioral Health Services	Administrative	Feb. 1, 2017
Breast Imaging for Screening and Diagnosing Cancer	Clinical	Feb. 1, 2017
Cardiology Procedures Requiring Precertification for eviCore healthcare Arrangement	Clinical	Feb. 1, 2017



UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates

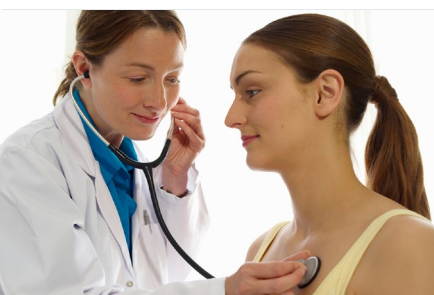
Title	Policy Type	Effective Date
Clotting Factors and Coagulant Blood Products	Clinical	Feb. 1, 2017
Co-Surgeon/Team Surgeon	Reimbursement	Feb. 1, 2017
Deep Brain and Cortical Stimulation	Clinical	March 1, 2017
Dental and Oral Surgical Procedures	Administrative	March 1, 2017
Diabetes Supply Coverage for Commercial Plans (Including New Jersey Small Group Plans)	Administrative	March 1, 2017
Drug Coverage Criteria - New and Therapeutic Equivalent Medications	Clinical	March 1, 2017
Drug Coverage Guidelines	Clinical	Feb. 1, 2017
		March. 1, 2017
Durable Medical Equipment, Orthotics, Ostomy Supplies, Medical Supplies and Repairs/Replacements	Administrative	March 1, 2017
Electrical Stimulation and Electromagnetic Therapy for Wounds	Clinical	Feb. 1, 2017
Eloctate™ (Antihemophilic Factor (Recombinant), FC Fusion Protein) for Connecticut Lines of Business	Clinical	Feb. 1, 2017
Emergency Room Visits (Including Coverage for Members Outside of the United States)	Administrative	Feb. 1, 2017
Entyvio® (Vedolizumab)	Clinical	Feb. 1, 2017
Experimental/Investigational Treatment	Clinical	Feb. 1, 2017
Experimental/Investigational Treatment for NJ Plans	Clinical	Feb. 1, 2017
Extended Benefits for Total Disability (Including Succeeding Carrier for Inpatient Admissions)	Administrative	Feb. 1, 2017
Formula & Specialized Food	Administrative	March 1, 2017
Gonadotropin Releasing Hormone Analogs	Clinical	Feb. 1, 2017
Helicobacter Pylori Serology Testing	Clinical	Feb. 1, 2017
Home Health Care	Clinical	March 1, 2017



UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates

Policy Title	Policy Type	Effective Date
Hysterectomy for Benign Conditions	Clinical	Feb. 1, 2017
Infertility Diagnosis and Treatment	Clinical	Feb. 1, 2017
In-Network Exceptions for Breast Reconstruction Surgery Following Mastectomy	Clinical	Feb. 1, 2017
Intravenous Enzyme Replacement Therapy (ERT) for Gaucher Disease	Clinical	Feb. 1, 2017
Maximum Dosage Policy	Clinical	Feb. 1, 2017
Maximum Frequency Per Day	Reimbursement	Feb. 1, 2017
Mechanical Circulatory Support Device (MCSD)	Clinical	Feb. 1, 2017
Mechanical Stretching Devices	Clinical	March 1, 2017
Multiple Procedures	Reimbursement	Feb. 1, 2017
Obstetrical Ultrasonography	Clinical	Feb. 1, 2017
Orencia® (Abatacept) Injection for Intravenous Infusion	Clinical	Feb. 1, 2017
Outpatient Physical & Occupational Therapy for Self-Funded Groups	Clinical	Feb. 1, 2017
Outpatient Physical and Occupational Therapy (OptumHealth Care Solutions Arrangement)	Clinical	Feb. 1, 2017
Oxford's Outpatient Imaging Self-Referral Policy	Clinical	Feb. 1, 2017
Physical, Occupational (OptumHealth Care Solutions Arrangement) and Speech Therapy including Cognitive/Neuropsychological Rehabilitation for New Jersey Small Group Members	Clinical	Feb. 1, 2017
Physician Extenders	Reimbursement	Feb. 1, 2017
Procedure and Place of Service	Reimbursement	Feb. 1, 2017
Prosthetic Devices, Wigs, Specialized, Microprocessor or Myoelectric Limbs	Clinical	Feb. 1, 2017
Radiation Therapy Procedures Requiring Precertification for eviCore healthcare Arrangement	Clinical	March 1, 2017



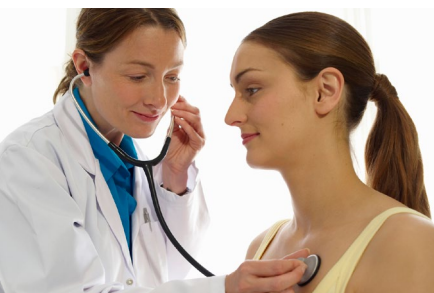
UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates

Policy Title	Policy Type	Effective Date
Radiology Procedures Requiring Precertification for eviCore healthcare Arrangement	Clinical	Feb. 1, 2017
Refractive Surgeries	Administrative	March 1, 2017
Repository Corticotropin Injection (H.P. Acthar Gel®)	Clinical	Feb. 1, 2017
Respiratory Interleukins (IL) Policy	Clinical	Feb. 1, 2017
Services and Modifiers Not Reimbursable to Healthcare Professionals	Reimbursement	Feb. 1, 2017
Soliris® (Eculizumab)	Clinical	Feb. 1, 2017
Speech Therapy and Early Intervention Programs/Birth to Three	Administrative	Feb. 1, 2017
Stelara® (Ustekinumab)	Clinical	Feb. 1, 2017
Timeframe Standards for Benefit Administrative Initial Decisions	Administrative	Feb. 1, 2017
REPLACED		
Diabetes Supply Coverage for New Jersey Small Group and Individual Plans	Administrative	March 1, 2017

Note: The inclusion of a service or procedure to this list does not imply that Oxford provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.

Oxford HMO products are underwritten by Oxford Health Plans (NY), Inc., Oxford Health Plans (NJ), Inc. and Oxford Health Plans (CT), Inc. Oxford insurance products are underwritten by Oxford Health Insurance, Inc.



UnitedHealthcare Affiliates

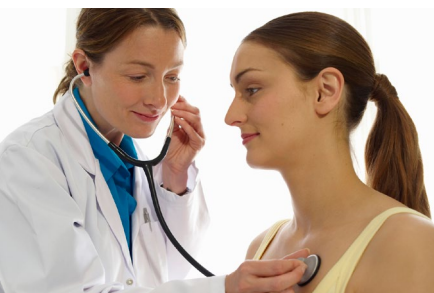
SignatureValue/UnitedHealthcare Benefits Plan of California Benefit Interpretation Policy



For complete details on the policy updates listed in the following table, please refer to the **February 2017 SignatureValue/UnitedHealthcare Benefits Plan of California Benefit Interpretation Policy Update Bulletin** at [UHCWest.com > Provider Log In > Library > Resource](#)

Policy Title	Applicable State(s)
REVISED (Effective March 1, 2017)	
Court, Attorney, or Agency Requested Services	All (California, Oklahoma, Oregon, Texas, & Washington)
Diabetic Management, Services and Supplies	All
Services While Confined/Incarcerated	All
Treatment of Morbid Obesity	Oklahoma, Oregon, Texas, & Washington

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.



UnitedHealthcare Affiliates

SignatureValue/UnitedHealthcare Benefits Plan of California Medical Management Guideline Updates



For complete details on the policy updates listed in the following table, please refer to the **February 2017 SignatureValue/UnitedHealthcare Benefits Plan of California Medical Management Guidelines Update Bulletin** at UHCWest.com > Provider Log In > Library > Resource Center > Guidelines & Interpretation Manuals.

Policy Title	Effective Date
UPDATED/REVISED	
Bariatric Surgery	March 1, 2017
Bone or Soft Tissue Healing and Fusion Enhancement Products	March 1, 2017
Deep Brain and Cortical Stimulation	March 1, 2017
Epidural Steroid and Facet Injections for Spinal Pain	March 1, 2017
Fecal Calprotectin Testing	Feb. 1, 2017
Home Hemodialysis	March 1, 2017
Omnibus Codes	March 1, 2017
Outpatient Cardiovascular Telemetry	Feb. 1, 2017
Surgical and Ablative Procedures for Venous Insufficiency and Varicose Veins	Feb. 1, 2017
Transcranial Magnetic Stimulation	Feb. 1, 2017
RETIRED	
Intermittent Intravenous Insulin Therapy	Feb. 1, 2017

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.